جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سميره محمد عبداللطيف سيد
المحافظة : القاهره **الحي / المدينة :** العتبه **العنوان :** --------
رقم البطاقة : 28210141301446 **رقم التليفون :** 01001882508 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 14-10-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سميره محمد عبداللطيف سيد
National ID: 28210141301446

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

نصيره
محمد عبد اللطيف سيد احمد

دمشا
مركز منشأة القناطر - الشرقية

٢٨٢١٠١٤١٣٠١٤٦

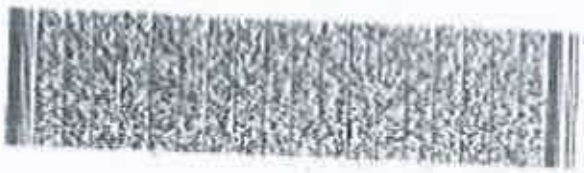
KB6562635

٠١٠٠١٣٨٢٥٠٨



٢٨٩١٠١٤١٣٠١٤٤٦ ٢٠٢٣/٠٨
حاصله على ليسانس
دراسات اسلاميه وعربيه
انثى مسلمة متزوجة
السيد رضوان السيد محمد
البطاقة سارية حتى ٢٠٣٠/٠٨/٠١

٨



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سميرة محمد عبدالعظيف سيد
Sex: Female
3- Medical number: 83218672
Old Med : 2221-2024
2-ID number:
Patient Age : 42 Y

MDT

Family history		Phone	01063037822	BMI		REASON OF MDT	POST OPERATIVE
marital status	MARRIED	NRS Score		GROUP	C	Date of MDT	01/09/2024
Co morbidity	-	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
Bilateral Multifocal breast ca

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT: Lt: lobular	8/8 ER COMMENT: Lt: 4/8	8/8 PR COMMENT: Lt: 7/8	0 Her 2 Neu COMMENT: Lt: 0	SISH COMMENT:	Rt: 30% Lt: 1%

T	N	M
Rt: cT2m Lt: cT2	Rt: cN1 Lt: cN0	0

Sonomammography
CESM: Rt Br Upper central , UIQ mass and non-mass enhancement all 6.8x6.5
Left Breast multifocal mass

PATIENT HISTORY
RT BCS+ALN:IDC GII with DCIS
+ve LN 7/16 with extra nodal extension , free margin+extra extra suprior & inferior , free other margi
LT BCS+SLN: IDC GII with DCIS
free LN 0/8 , free extra deep margim & other margin

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
- BRCA test
- Start 4 AC Followed by Taxotere *4 cycles - Hormonal Treatment post-operative RTH - Zoladex

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. هنان عزت د. رضوى هلال	RADIOLOGY	RADIOLOGY د. شريف سامح
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: سميره محمد عبداللطيف سيد **MR. :** 9894	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with bilateral breast cancer and underwent bilateral breast-conserving surgery on 3/8/2024. Pathology of the right breast revealed a pT2 N2 tumor, with estrogen receptor (ER) positivity in 7/8 cores, progesterone receptor (PR) positivity in 7/8 cores, HER2-negative status, and Ki-67 of 15%. The left breast pathology showed a pT2 N0 (0/5 nodes) tumor, with ER positivity in 5/8 cores, PR positivity in 7/8 cores, HER2-negative status, and Ki-67 of 1%. The patient subsequently received a 4-cycle AC (doxorubicin/cyclophosphamide) regimen followed by 4 cycles of taxane therapy, a port placement, and initiation of adjuvant endocrine therapy with anastrozole (Arimidex) and leuprolide acetate (Zoladex) starting in February 2025, with planned continuation of the hormonal agents for three months.

On examination the patient reported vaginal spotting; serum estradiol (E2) level was within normal limits. A dilation and curettage (D&C) was performed, revealing an endocervical mucosal benign polyp. The patient continues on anastrozole and leuprolide acetate for a planned three-month course, with further management pending.

تحريرا في ,

التاريخ :	**18-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Patient Name	: [illegible]	Request No.	: 13007911
Patient No.	: 28210141301446	Request Date	: 01/08/2024
Path No.	: SR-1056/24	Verification Date	: 10/08/2024
Sex	: Female	Financial Category	: برنامج دار السلام - بلدية الدولة
Age	: 41 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:
- Two paraffin blocks labeled by (18755) and Ten slides were received.

Microscopic Examination:
- Right breast
* Histological type: Invasive duct carcinoma
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Could not be identified
* Angioinvasion: Could not be identified
* Perineural invasion: Could not be identified
* Microcalcifications: Could not be identified
* IPT:
- ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 15% nuclear reaction of tumor cells.

- Left breast
* Histological type: Invasive duct carcinoma with focal lobular component
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Could not be identified
* Angioinvasion: Could not be identified
* Perineural invasion: Could not be identified
* Microcalcifications: Could not be identified
* IPT:

Verified by :
Dr. Sagda El-Kady

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	:	Request No.	: 13007911
Patient No.	: 282101411301446	Request Date	: 01/08/2024
Path No.	: SR-1056/24	Verification Date	: 10/08/2024
Sex	: Female	Financial Category	:
Age	: 41 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

- ER: Moderate nuclear reaction in 20% of tumor cells (score 5/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 1% nuclear reaction of tumor cells.

Diagnosis:
Bilateral breast cancer, Slide revision:
- Right breast : Invasive duct carcinoma, Grade II
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 15% of tumor cells

- Left breast : Invasive duct carcinoma with focal lobular component, Grade II
IPT:
- ER: Positive (score 5/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 1% of tumor cells

Signature
Prof. Dr. Amal Hareedy
Dr. Nermin Mohamed

Verified by :
Dr. Sagda El-Kady

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سميره محمد عبد اللطيف	Request No.	: 13008317
Patient No.	: 28210141301446	Request Date	: 03/08/2024
Path No.	: 2716/24	Verification Date	: 21/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

(Amended Report)
Clinical Data : Bilateral breast cancer
Nature of the specimen : Right BCS & Right ALN biopsy &Left CBS&left SLNB

Gross Examination:
- Procedure: Right BCS specimen measured 11x8x2 cm , no covering skin ellipse
- Tumor extent : A well defined greyish white firm mass measured 3.5x2.5x2.5 cm .
- Distance from margins: 0.2 cm away from deep margin
- Right superior margin : Fibrofatty tissue piece 4.5x3.5x2.5 cm
- Right extra inferior margin : fibrofatty tissue piece 3.8x2.8x0.5
- Right extra extra superior margin : Fibrofatty tissue piece 3.5x2x2 cm
- Right extra extra inferior margin : fibrofatty tissue piece 3x5x2 cm
- ALND: Fibrofatty tissie piece 7.5x6x3 cm entangling 16 nodules
- Frozen section: Suspicious deep, inferior , superior , extrasuperior, extra inferior ,Free other margins
- Procedure: Left BCS specimen measured 10x9x2 cm ,No covering skin ellipse
- Tumor extent : A well defined greyish white firm mass measured 2.5x2x1.5 cm .
- Distance from margins: 0.1 cm away from deep margin
- Lymph nodes number and size: A fibrofatty tissue piece measured 5x4x1 cm enclosing 5 SLNs up to 1.5x1 cm .
- Exra deep : Fibrofatty tissue piece 7.5x3x1 cm
- Frozen section: involved deep margin , Free other margins & Free SLNs (0/3)

Microscopic examination:
Right :
- Histological type:Bifocal Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G III(3+3+2)
- Angioinvasion: No
- Ductal carcinoma in situ: present

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سميره محمد عبد اللطيف	Request No.	: 13008317
Patient No.	: 28210141301446	Request Date	: 03/08/2024
Path No.	: 2716/24	Verification Date	: 21/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Lymph node involvement: Free SLNs (0/5)
- Resection Margins: Free extra extra superior, extra etxra inferior, free other margins
- Lymph nodes : positive (7/16) with extra nodal extension
- Additional pathologic findings: Microcalcification, Another microscopic focus of invasive tumor measured 0.3x0.3 cm detected in inferior margin with free extrainferior margin.

Left side :

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G III(3+3+2)
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: Free SLNs (0/5)
- Resection Margins: Free extra deep margins& free other margins

Diagnosis: (Revised)

Right BCS & Right ALN biopsy:

Invasive duct carcinoma Grade III with DCIS

- Positive LNs (7/16) with extra nodal extension
- Free margins (Free extra extra superior &inferior , free other margins)
- Stage : pT2m, pN2a

Left BCS & left SLN biopsy:

Invasive duct carcinoma Grade III with DCIS

- Free LNs (0/5)
- Free extra deep margin, free other margins
- Stage : pT2 , pN0

Prof. Dr. Gamal Abdelatty

Dr.Menna Nabil

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: سميره محمد عبداللطيف سيد احمد	Study Date: 20-Jul-24
PROF: .Ahmed Aly Obaya	Patient ID: 649614

18F-FDG PET/CT EXAMINATION

History:-

40 years old female patient, presented with history of bilateral breast masses [pathologically proven as infiltrating duct carcinoma] and referred for PET/CT assessment.



Procedure:-

- 4.4 mCi 18F-FDG was a dministrated to the patient and imaged after appropriate time.
- The blood glucose level was 112 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault till thighs were obtained.
- A diagnostic 128 MDCT non contrast examination was taken of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

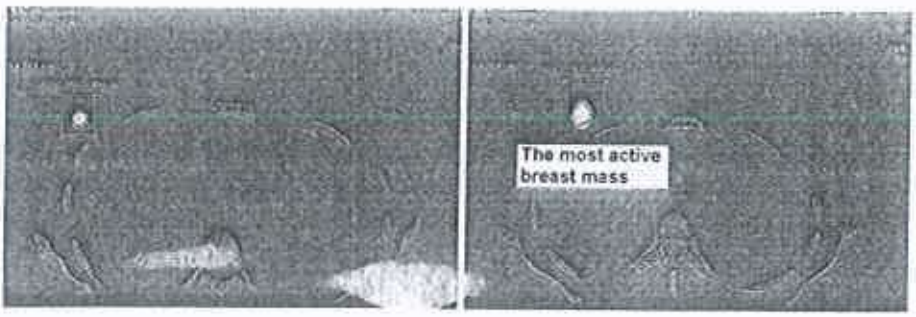
PET/CT FINDINGS:-

A] Head and Neck:-

- No hypermetabolic lymph nodes.
- The visualized portions of the brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- No parenchymal thyroid focal lesions could be elicited.

B] Chest:-

- **Few ill-defined bilateral hypermetabolic soft tissue m asses are noted at both breasts noted as follows :**



PATIENT NAME: SAMIRA MOHAMMED ABD EL LATEEF SAYED
GENDER: Female
DATE : 23-Jul-2024 11:42 AM

PATIENT ID: 10124245858
AGE: 41 y
ORDER: 2955291,2955757

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV non-ionic contrast medium infusion 100 ml of ultravist

REPORT:

- Contrast Enhanced Spectral Mammography showed minimal symmetrical back ground parenchymal enhancement.

Right breast:

- Upper central and UOQ irregular spiculated mass showing intense heterogenous mass enhancement with anterior faint heterogenous segmental non mass enhancement, collectively measured 6.8x6.5x6.8cm cm, reaching retroareolar region, being just about 0.9cm from the nipple and about 1.8cm deep to the skin.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Pathologically enlarged right axillary lymph node.

Left breast:

- Upper central irregular mass showing intense heterogenous enhancement measured 2.6x2.7x2.1cm with small anterior adjacent satellite, being 1.9 cm from the nipple.
- Another spiculated mass with moderate heterogenous enhancement seen superio-lateral to the index mass (in UOQ) with inter-lesional distance about 2.5cm. It measures about 2.9x2.3x2.3cm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Prominent left axillary lymph node.

OPINION:

- Right breast upper central and UIQ pathological mass and non mass enhancement (BIRADS 6).
- Left breast multifocal pathological mass enhancement (BIRADS 6).

www.Baheya.org
16602

Mammogram
[[DOCTYPE:MAMMOGRAM]]

CONCLUSION:-

- **Positive PET/CT study for bilateral few hypermetabolic neoplastic breast masses { pathologically proven as invasive duct carcinoma} with solitary loco-regional metabolically active right axillary metastatic lymphadenopathy , otherwise, no distant metastases.**
- **Post therapeutic follow is recommended**

MUCH OBLIGED
Dr. Mohamed Hesham Saleh , MD
PROF.DR. MAGED ABDEL-GALIL

E-Signed Maged Abdelgalil